

Turn scattered safety records into a **decision-ready** knowledge base



The Problem

Your safety knowledge — incident reports, near-miss memos, complaint records, committee minutes, improvement plans — is scattered across reporting systems, ward files, and meeting folders, and locked in your senior nurses' and safety officers' heads. Finding whether a similar incident occurred before takes days; prevention depends on institutional memory.



What Neuron Does

It links every record into one ontology-structured database anchored to each incident and case ID, so your team gets instant answers to "what conditions preceded similar incidents?" and "which wards and procedures share this risk?" — without changing how you work today.

What you can do with the centralized database

01

Detect harm signs before adverse events

Flag near-miss clusters that trend toward past serious-incident patterns — same drug pair, same handoff, same time pressure — so intervention happens before harm.

02

Check risk across wards and procedures

When one incident occurs, instantly see every ward, procedure, and staffing pattern sharing the same conditions and judge the full exposure.

03

Verify countermeasures actually worked

Track whether the same incident class recurred after each protocol change — evidence you can show the safety committee and accreditation surveyors.

04

Make safety decisions with full context

Surface similar past incidents and what stopped them at review time, so committees decide on evidence, not anecdote.

"The same near-miss pattern had been reported in three different wards over two years — and no one connected them. Now the connection surfaces the day the third report comes in."

Patient Safety Officer - General hospital

See it with your own data

We start with a focused PoC using your existing safety records — incident reports, committee minutes, improvement plans, as they are. No system migration, no process change required.

Let's talk for 30 minutes. →

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